

1. Administrative & Study Identification

Full Study Title: An Interventional, Multicentre, Phase IV, Single-Arm, Open-Label Study to Investigate the Efficacy and Safety of Dapagliflozin in Chinese Adult Patients With Chronic Kidney Disease **Short Title/Acronym:** A Study of Dapagliflozin in Chinese Adult Patients With Chronic Kidney Disease **Protocol Number:** D169AC00008 **NCT Number:** NCT06610526 **Trial Status:** Recruiting (Currently recruiting participants) **Sponsor Name:** AstraZeneca **Investigational Product:** Dapagliflozin (SGLT2 inhibitor; ATC Code: A10BK01; Trade Names: Qternmet, Qtern, Xigduo, Farxiga) **Phase of Development:** Phase IV **Medical Monitor:** AstraZeneca Clinical Operations Lead

2. Study Synopsis & Rationale

2.1 Study Objective Primary Objective: To investigate the efficacy and safety of dapagliflozin to prevent the progression of chronic kidney disease in Chinese adult patients. **Secondary Objectives:** To describe general efficacy and safety outcomes associated with dapagliflozin in Chinese patients with chronic kidney disease.

2.2 Background & Rationale To address the gap in real-world clinical data and describe the efficacy and safety information with dapagliflozin in Chinese patients with chronic kidney disease (CKD). Generating this Phase IV data will help reinforce the clinical benefits of dapagliflozin in preventing disease progression and reducing the risk of renal and cardiovascular mortality within this specific population.

3. Study Design & Methodology

3.1 Design Framework Study Type: Interventional **Control Type:** Single-arm **Blinding:** Open-label **Randomization:** N/A

3.2 Planned Enrollment & Duration Target \$N\$: 700 evaluable adult Chinese patients **Number of Sites:** Multicentre (across China) **Estimated Study Start:** 23-Aug-2024 **Estimated Primary Completion:** 31-May-2027

4. Subject Selection (Eligibility Criteria)

4.1 Inclusion Criteria 1. Provision of signed informed consent prior to any study specific procedures. 2. Chinese Female or male

aged ≥ 18 years at the time of consent. 3. Confirmed diagnosis via eGFR ≥ 25 and ≤ 75 mL/min/1.73m² (CKD-EPI Formula) at visit 1. 4. Evidence of increased albuminuria 3 months or more before visit 1 and UACR ≥ 200 and ≤ 5000 mg/g at visit 1. 5. Stable, maximum tolerated labelled daily dose treatment with ACE-I or ARB for at least 4 weeks before visit 1, if not medically contraindicated.

4.2 Exclusion Criteria 1. Autosomal dominant or autosomal recessive polycystic kidney disease, lupus nephritis or anti-neutrophil cytoplasmic antibodies-associated vasculitis. 2. Receiving cytotoxic therapy, immunosuppressive therapy or other immunotherapy for primary or secondary renal disease within 6 months prior to enrolment, or history of organ transplantation. 3. Receiving therapy with an SGLT2 inhibitor within 8 weeks prior to enrolment or previous intolerance of an SGLT2 inhibitor. 4. Type 1 Diabetes Mellitus (T1DM); New York Heart Association class IV Congestive Heart Failure at enrolment; MI, unstable angina, stroke or TIA within 12 weeks prior to enrolment. 5. Coronary revascularization (PCI or CABG) or valvular repair/replacement within 12 weeks prior to enrolment. 6. Any condition outside the renal and CV disease area, such as but not limited to malignancy, with a life expectancy of less than 2 years based on investigator's clinical judgement. 7. Active malignancy requiring treatment at the time of visit 1 (with the exception of successfully treated basal cell or treated squamous cell carcinoma). 8. Hepatic impairment (AST or ALT $> 3 \times$ ULN; or total bilirubin $> 2 \times$ ULN at time of enrolment). An isolated increase in bilirubin in patients with known Gilbert's syndrome is not a reason for exclusion. 9. Known blood-borne diseases. 10. Women of child-bearing potential who are not willing to use a medically accepted method of contraception, OR women who have a positive pregnancy test at enrolment OR women who are breast-feeding. 11. Involvement in the planning and/or conduct of the study, previous enrollment in the present study, or participation in another clinical study with a study intervention during the last month prior to enrolment. 12. Inability of the patient to understand and/or comply with study intervention, procedures and/or follow-up.

5. Intervention & Treatment Plan

5.1 Investigational Regimen Dosage/Frequency: Dapagliflozin 10 mg once daily. **Route of Administration:** Oral. **Duration of Treatment:** Up to a median follow-up time of 24 months (estimated maximum around 33 months).

5.2 Concomitant & Prohibited Therapy Permitted: Stable use of ACE-I or ARB therapy at the maximum tolerated labelled daily dose. **Prohibited:** Cytotoxic therapy, immunosuppressive therapy, or treatment with other SGLT2 inhibitors.

6. Study Timeline & Visit Schedule

Visit ID	Window (Days)	Key Activities/Procedures
Screening	Day -14 to 0	Informed Consent, Medical History, eGFR/UACR Labs
Baseline	Day 0	First Dose, Patient Baseline Assessment, Quality Audit
Interim	Scheduled Follow-ups	Safety Labs, Efficacy Assessment (eGFR tracking), CV/Renal Event Monitoring
End of Study	Up to Month 33	Final Vital Signs, Exit Interview, IP Accountability, Final Endpoints

7. Outcome Measures (Endpoints)

7.1 Primary Endpoint Definition: Time to the first occurrence of any of the components of the composite: 50% sustained decline in eGFR, or Reaching End-Stage Renal Disease (ESRD), or Cardiovascular (CV) Death, or Renal Death. **Measurement Method:** Clinical assessment and Central/Local Laboratory Analysis; data reported as descriptive statistics.

7.2 Secondary Endpoints 1. Percentage change in UACR from baseline. 2. Time to the first occurrence of any of the components of the composite: $\geq 50\%$ sustained decline in eGFR or Reaching ESRD or Renal death. 3. Time to the first occurrence of either of the components of the composite: CV death or Hospitalization for heart failure.

8. Safety & Adverse Event Reporting

Adverse Event (AE) Monitoring: Standard collection via patient reports, laboratory findings, and HCP interviews throughout the follow-up period. **Serious Adverse Events (SAEs):** Reported within 24 hours of site awareness, particularly focusing on severe renal or cardiovascular events. **Data Monitoring Committee (DMC):** Internal safety monitoring mechanisms managed by the Sponsor (AstraZeneca).

9. Statistical Analysis Plan (SAP) Summary

Analysis Populations: Intent-to-Treat (ITT) population for primary effectiveness outcomes; Safety Set for all AE/SAE evaluations. **Sample Size Justification:** $N=700$, appropriately powered to evaluate descriptive statistics on the composite time-to-event renal and cardiovascular outcomes. **Handling of Missing Data:** Standard statistical models for time-to-event analysis (e.g., Kaplan-Meier, Cox proportional hazards) censoring dropouts where applicable.

10. Quality Assurance & Regulatory Compliance

GCP Compliance: This study will be conducted in accordance with Good Clinical Practice (GCP) and the Declaration of Helsinki.

Monitoring Plan: Regular onsite and remote monitoring visits per standard sponsor operating procedures. **Audit Trail:** Details on eCRF locking, query resolution, and data clarification forms via centralized mechanisms.

11. Study Identifiers & Governance

Primary ID: D169AC00008 **NCT Number:** NCT06610526 **Posting ID:** 381375159678120420 **Sponsor/Lead Organization:** AstraZeneca **Study Title:** A Study of Dapagliflozin in Chinese Adult Patients With Chronic Kidney Disease **Official Regulatory Title:** An Interventional, Multicentre, Phase IV, Single-Arm, Open-Label Study to Investigate the Efficacy and Safety of Dapagliflozin in Chinese Adult Patients With Chronic Kidney Disease **Company:** AstraZeneca

12. Clinical Framework (CKD Specific)

Primary Condition: Chronic Kidney Disease (CKD) **Diagnosis Threshold:** eGFR ≥ 25 and ≤ 75 mL/min/1.73m²; UACR ≥ 200 and ≤ 5000 mg/g **Medical Methodology:** Guideline Determined Medical Therapy (GDMT) with SGLT2 inhibitor addition **Optimization Target:** Prevention of 50% sustained decline in eGFR, ESRD, or CV/Renal death

13. Study Procedures & Methodology

Management Pathway: Standard clinical pathway for CKD progression prevention **Education Protocol (HCP):** Site visits and training regarding SGLT2i integration into CKD management **Education Protocol (Patient):** Medication adherence instructions, dietary control, and self-learning materials **Quality Audit Mechanism:** HCP protocol adherence tracking & patient adherence monitoring

14. Observation & Follow-Up Schedule

Total Duration: Up to 33 Months (Median follow-up of 24 months) **Onsite Visit Cadence:** Baseline, and protocol-defined interim follow-ups **Remote Monitoring Frequency:** Periodic tele-visits if required **Checklist Review Interval:** Standard intervals (e.g., Q12W) for drug compliance

15. Sign-off & Approval

Role	Name	Signature	Date						
Principal Investigator	[Name]	_____	[DD-MMM-YYYY]						
Clinical Operations Lead	[Name]	_____	[DD-MMM-YYYY]						
Statistician	[Name]	_____	[DD-MMM-YYYY]						